

Language Access and Interpreter Services

Policy Number	POL-CMP-084	Owner / Department	Patient Experience / Compliance
Effective Date	2024-02-01	Version	2.1
Last Revised	2026-04-20	Next Review	2028-04-20
Approved By	Patient Experience Committee	Applies To	All staff and providers serving patients and representatives with limited English proficiency or who are deaf or hard of hearing

1. Purpose

This policy ensures meaningful access to care for patients with limited English proficiency (LEP) and for patients who are deaf or hard of hearing, consistent with Title VI of the Civil Rights Act, Section 1557 of the Affordable Care Act, and the national CLAS standards. Qualified interpreter services are provided free of charge.

2. Policy

- Offer and provide a qualified interpreter at no cost for all significant clinical encounters.
- Do not require or rely on family members, friends, or minors to interpret; a minor may interpret only in a genuine emergency until a qualified interpreter is available.
- Do not rely on bilingual staff to interpret unless they are assessed and qualified to do so.
- Document the patient's preferred language and the interpreter modality used in the record.

3. Interpreter Access Modalities

Modality	Best for	How to access
In-person interpreter	Complex, sensitive, or high-stakes encounters (consent, end-of-life)	Schedule via Patient Experience; on-call roster after hours
Video remote interpreting (VRI)	Most encounters, including American Sign Language when in-person is unavailable	Use the unit VRI tablet; dial the language
Telephonic interpreting	Brief or urgent exchanges; less common languages	Dual-handset phone; call the vendor line and give the account code

IMPORTANT — For any informed consent, procedure, discharge instruction, or end-of-life discussion, a qualified interpreter must be used before the encounter proceeds. Do not proceed with a family member interpreting. If no interpreter can be reached, pause non-emergent care and escalate to the house supervisor.

4. Auxiliary Aids and Written Materials

- Provide auxiliary aids (ASL interpreters, captioning, assistive listening) for patients who are deaf or hard of hearing.
- Post the facility's notice of nondiscrimination and taglines in the top languages of the service area.
- Translate vital documents (consent, rights, discharge instructions) into the most frequently encountered languages.

5. Severity & Reporting

Failures of language access are classified on the facility-wide scale in POL-RM-013. A communication failure that contributed to serious harm (for example, an invalid consent for a procedure) is SEV-1 (notify the Administrator-on-Call and Risk Management within 1 hour). A major access failure without harm is SEV-2 (notify Patient Experience within 4 hours). A minor lapse or a Near Miss is SEV-3 / Near Miss (report within 24 hours).

6. References

- Title VI of the Civil Rights Act of 1964; Section 1557 of the Affordable Care Act (45 CFR Part 92).
- National CLAS Standards (HHS Office of Minority Health).
- Related: POL-CMP-083 (Patient Rights), POL-RM-013 (severity & reporting).